1} Antenatal Assessment

Physical examination during pregnancy

Goal:

This activity will increase nursing skills, knowledge and confidence of the student nurse in assessing pregnant women through a better understanding of performing systematic physical assessment of the pregnant woman.

Objectives:

By the end of this procedure, the student nurse should be able to:

1. Prepare the appropriate equipment needed for doing physical examination of pregnant woman
2. Describe the component of the pre-natal physical examination.
3. Use a systematic and developmentally appropriate approach for examining pregnant woman.
4. Describe normal physical examination findings.
5. Identify the Laboratory tests performed during pregnancy.

Outlines:

- Introduction
- Definitions
- Objectives
- Equipment
- Components of the pre-natal physical examination
- Pre-requisites (Principles)

Introduction:

The prenatal period is the preparatory one, both physically, in terms of fetal growth and maternal adaptations, and psychologically, in terms of anticipation of parenthood. Regular prenatal visits, ideally beginning soon after the first missed menstrual period, offer opportunities to ensure the health of the expectant mother and her infant. Antenatal history taking and assessment is one of the important components of the antenatal care

Definitions:

- **Antenatal care:**

  Refers to the care given to an expectant mother from the time the conception is confirmed until the beginning of labor.

- **Physical examination during pregnancy:**

It is an assessment done in a detailed systematic (top to toe) order when a pregnant woman attends the antenatal clinic for the 1st time. https://images.app.goo.gl/NtkCQvX6N9tRk7VJ7

Objectives:

- **Overall objective:**

  It is to ensure a normal pregnancy with delivery of a healthy baby from a healthy mother.

- **Specific objectives:**

  1. To assess the health status of the mother and the fetus, to formulate the plan of subsequent management.

2. To monitor the growth and development of the fetus.
3. To calculate the estimated date of birth.
4. To obtain baseline information against which the subsequent changes are assessed and which are of importance in the determination of the gestational age.
5. To screen the "high risk" cases.
6. To prevent or to detect and treat at the earliest any untoward complications.
7. To ensure continued medical surveillance and prophylaxis.

Equipment:

1. Stethoscope,
2. Light, measuring device.
3. Thermometer
4. Sphygmomanometer.
5. Tongue depressor
6. Weighing scale
7. Uterine testing facility (ultrasound)
8. Client record.

Components of the pre-natal physical examination:

Pre-natal care begins with a positive pregnancy test. The first visit is the most important visit for the mother and the nurse, in which the mother is fully identified. It is done through 3 steps; the first step is taking the history and in this step 9 dates are taken; the second step is performing an abdominal examination and the third step is laboratory tests.

Pre-requisites (Principles)

1. Wash hands before and after the procedure to prevent cross infection.
2. Proper explanation of the procedure should be given to the mother to gain cooperation.

3. Ask the mother to evacuate her bladder.
4. Mother should be placed in comfortable position (dorsal position with slightly flexed knees) and should be convenient for the nurse.
5. Maintain privacy by providing screen or conduct in a private room if available.
6. Abdomen should be fully exposed.
7. Warm hands by rubbing it with each other before touching the abdomen and hands should not be removed until the procedure is over.
8. Provide good source of light.
9. Collect required articles at bedside and stand on the right side of the mother.
10. Recording and reporting to be done.

Antenatal care first Step: -

1} Take History:

In this step, the mother is asked about 8 important histories. They are asked about during the first visit only and will not be repeated, so you must be careful in this step.

1- Personal Information:

- ☞ Name *Age
- ☞ Marital status * Type of work
- ☞ Level of education

2- Menstrual history:

- First day of the last normal menstrual period **(LMP).**
- Calculation of gestational age, and expected date of delivery **(EDD)**.

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3- Obstetric history:

- N.o of pregnancy (Gravida)
- N.o of labor (Para)
- Antepartum care, labor, and puerperium of previous pregnancies • Mode of delivery
- Number and sex of living children • Birth weights
- Mode of infant feeding.
- Date of last labor and last abortion (LL and LA)

4- Present pregnancy history:

- Symptoms of pregnancy
- Symptoms of pre-eclampsia
- Symptoms of disease in other organ systems • Fetal movements

5- Present Complaints:

- In detail and duration

6- Family history

- Diabetes mellitus Hypertension
- Multiple pregnancies Congenital anomalies

7- Medical history:

- Chronic diseases - Diabetes mellitus - Hypertension
- Urinary tract infections Heart diseases - Viral infection

8- Drugs/ allergies.

9- Surgical history:

- Dilation and curettage . Vaginal repair
- Cesarean section
- Non-Gynecologic operations

10- Family planning history and type of work (If found)

11- Vital signs and measure height and weight

- It is important, after taking the history and before performing a local examination, to take vital signs and measure height and weight.
- This step is repeated in each visit until delivery.

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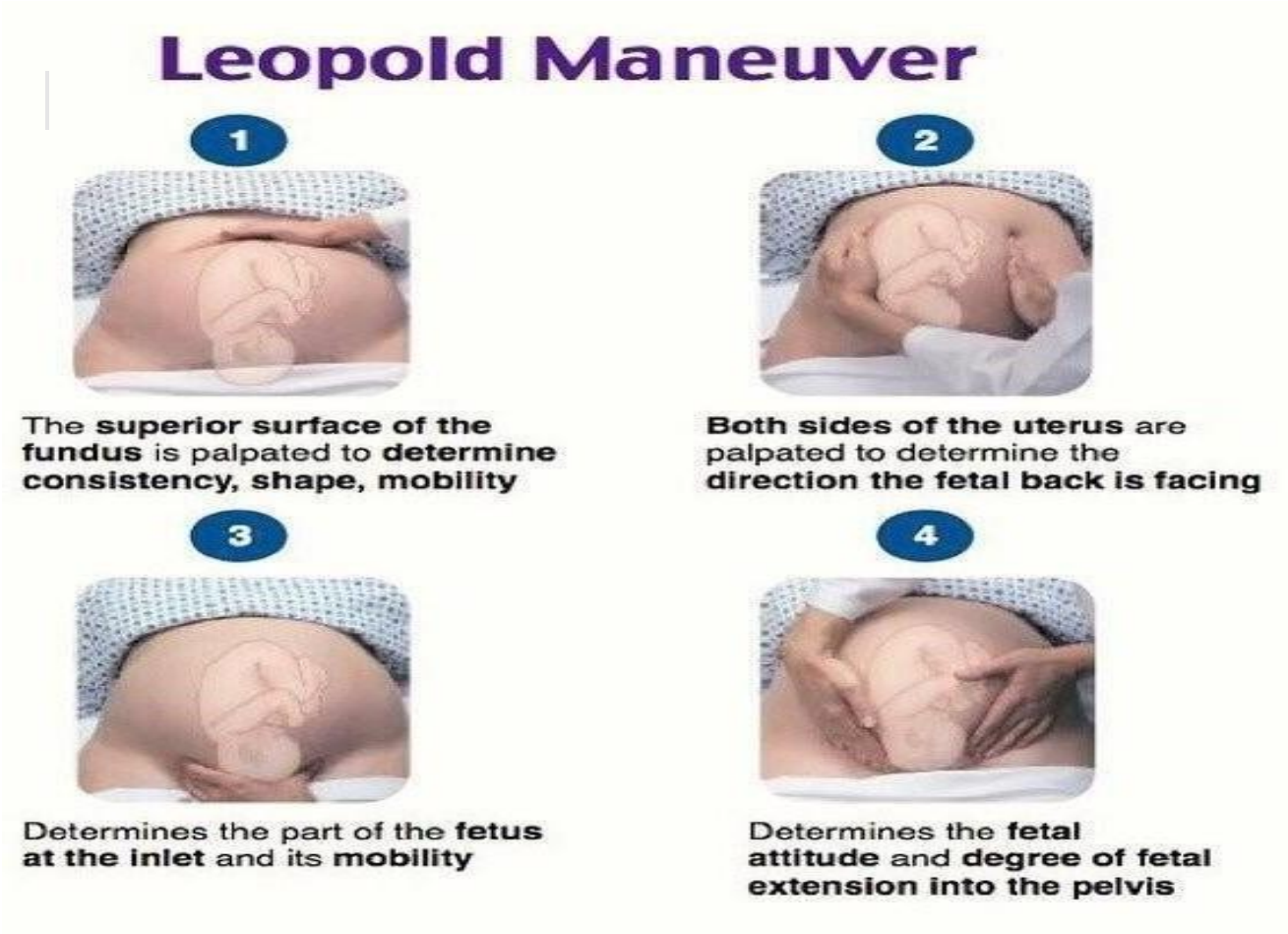
Step (2): Local examination (Abdominal Examination):

1. Inspection:

- Contour and size of abdomen
- Scars of previous operations
- Signs of pregnancy
- Fetal movements.

2. Palpation: (LEOPOLD MANEUVERS)

- Fundal level (FL)
- Fundal grip
- Umbilical grip
- Pelvic grip



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3-Auscultation:

Fetal Heart Sounds (FHS)

At 10 weeks, by Sonic aid

At 20 weeks, by Pinard's fetal stethoscope

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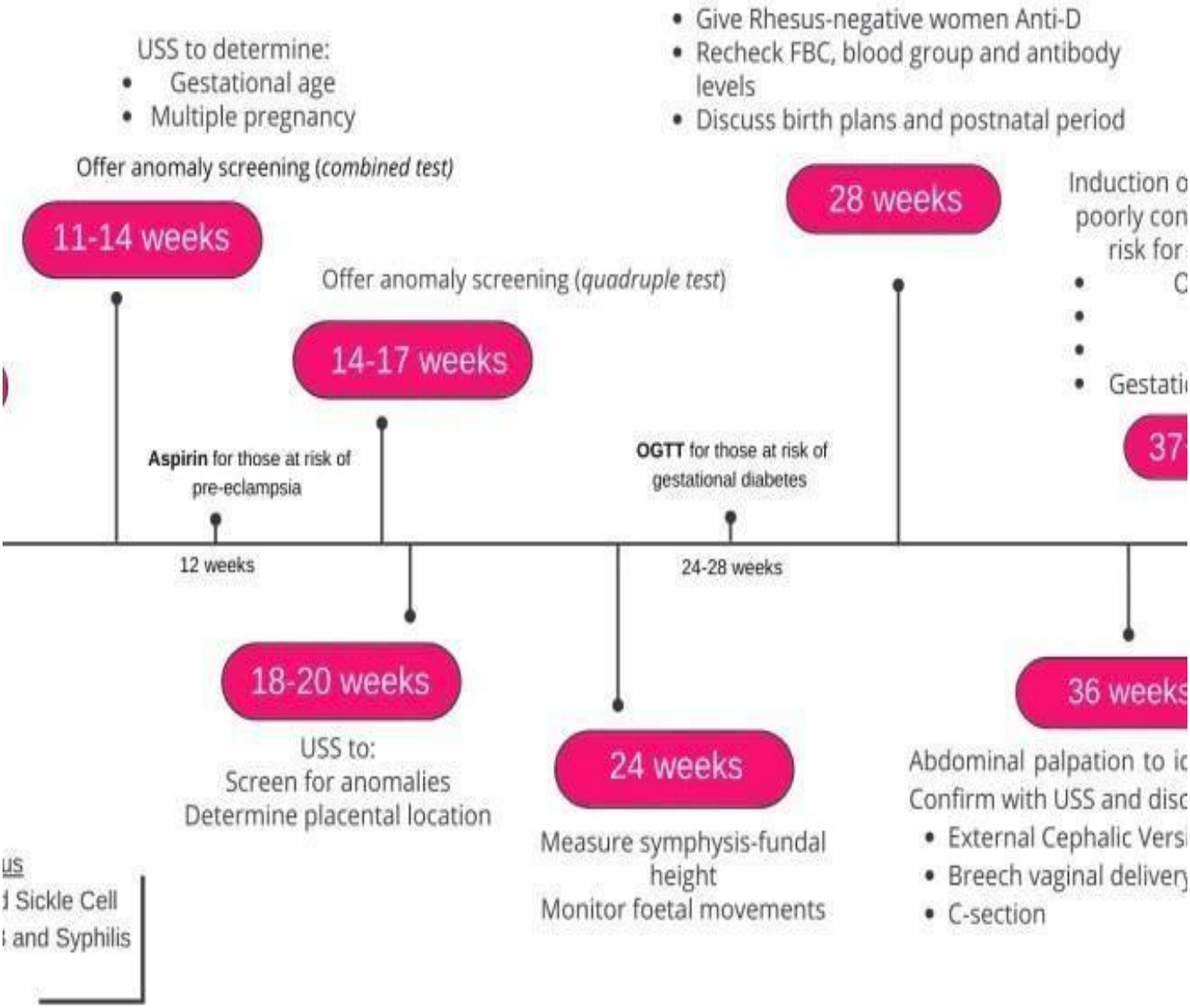
Step (3) Laboratory investigations:

Test	Rational
1. Urine analysis	☞ Screen for diabetes, evaluate at each visit.
2. Sugar	Screens for pre-eclampsia, evaluate for renal problems.
3. Albumin 4. Urine culture 5. Blood urea.	☞ Diagnose urinary tract infection. ☞ Evaluate renal function and diagnosis of renal diseases.
6. Blood tests 7. Complete blood count.	☞ Screens for anemia, iron deficiency.
8. White blood cells.	☞ Identified infectious processes.
9. Rh factor	☞ Alerts care provider to possible incompatibility disease in fetus.
10. Others ☞ Pap smear ☞ Culture of cervical discharge.	☞ For early Detection of cervical cancer. ☞ For diagnosis of sexually transmitted diseases.

Antenatal Care Timeline



https://teachmeobgyn.com/pregnancy/antenatal/antenatal-care/

1} Physical Examination during Pregnancy (Ante-Natal Physical Examination)

Action	First Trial		Second Trial	
	Done	Note Done	Done	Note Done
1. Prepare equipment				
2. Wash handes				
3. Welcome the woman, addressing her by name. and Introduce yourself with name & job title				
4. Explain the procedure				
5. Instruct her to empty the bladder and collect a midstream specimen of urine.				
6. Test urine for protein, sugar and Ketone				
7. Measure accurately woman's weight without shoes				
8. Measure accurately her hieght without shoes				
9. Measure correctly her blood pressure				
10. Measur corredtly her pulse				
11. Place the woman on the examination couch on her back				
12. Provid privacy				

13. Stand at the right side of the woman				
Examine head , face and neck				
14. Dandruff& hair loss				
15. Face (skin pigmentation "chlosma				
16. Eye (color of sclera, discharge & inflammation)				
17. Nose (epistaxsis & nasal congestion)				
18. Mouth (lips (color), gum (gingivitis), teeth (teeth decade)				
19. Neck , Thyroid , or any abnormality				
Chest & breast:				
20. Assisst with examination of the heart and lungs (apical puls& chest sound).				
21. Examine the breasts (size, pigamentation, clostrum, secondry nipples and areola).				
Abdomen				
22. Line nigra &stria gravidarum&hernia				
23. Presence of abdominal scar , abdominal edema				
Examine the extremities.				
➢ Edema: ☞ Extensiveness of edema, ☞ Depth of depression ☞ Length of time it takes to clear.				

☞ Grade the pitting edema ➢ Varicosities				
Genital Examination				
24. Vaginal discharge (colour, odour, amount) &vaginal bleeding or leakage of amniotic fluid.				
25. Anus &rectum (constipation, haemorrhoid)				
26. Assisst with pelvic examination.				
27. Check the woman for danger signs of pregnancy.				
Post Procedure				
28. Help the woman get down from the examination table and to put on her clothes.				
29. Record all findings and the woman's reaction to the exam.				
30. Return equipment				
31. Wash hands.				
32. Give the woman the necessary instructions and the date of next visit.				
33. Refer any abnormal finding				
Procedure score				
Date				

Student name: **Instructor name:**